NCCP 00485 · clinician draft

# Dose-dense AC → weekly paclitaxel

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Phase 1 · dose-dense AC

**Active medicines:** Doxorubicin + Cyclophosphamide

**Emetogenic risk:** High · AC

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row |
| Olanzapine | 5 mg orally | Days 1–4<br>Review sedation, falls, interactions and glucose |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications |

### Additional phase-specific support

- G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan.
- This script applies only to AC. It is replaced when weekly paclitaxel starts.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00485 v6 · p5 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-17 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required